

PATIENT AML004 - SOAP NOTE

Hem/Onc Clinic Note Date: 05/18/2023

S: Emily Jones (*9/1/72) diagnosed with adverse-risk AML (FLT3-ITD+, TET2+, SRSF2+) on 05/10/2022 s/p 7+3+midostaurin (mid 5/2022), SCT (11/2022), now D+180 post-transplant relapse. Increasing fatigue x2wks. Bruising easily. No fevers/chills. Mild dyspnea on exertion.

O: VS: 98.2, 88, 18, 108/72, 98% RA Gen: Pale, tired-appearing HEENT: Conjunctival pallor CV: RRR, no m/r/g Pulm: CTAB Abd: Soft, spleen tip palpable Ext: Scattered ecchymoses Neuro: AAOx3

Labs:

- WBC 18.7 (65% blasts on diff)
- Hgb 6.8
- Plt 31K
- Cr 0.9
- LDH 512
- Uric acid 6.8

BM Bx (5/15/23): Hypercellular, 73% blasts. FLT3-ITD detected (ratio 0.82).

A: 50F with relapsed FLT3+ AML, 12mo post initial tx, 6mo post-allo SCT. Aggressive disease biology with early post-transplant relapse.

P:

1. Discussed prognosis - median survival 3-4mo with relapsed post-transplant AML
2. Offered gilteritinib 120mg daily - pt agreed to try
3. Transfuse 2u PRBC today, plt if <10K
4. Allopurinol 300mg daily
5. F/u 1 week with CBC
6. Palliative care referral placed
7. Code status discussion - remains full code currently

Mark Stevens, MD

[Addendum 06/22/2023: Patient admitted 06/19/23 with septic shock, multiorgan failure. Despite aggressive measures including mechanical ventilation and pressors, patient expired 06/21/2023 at 03:45. Family at bedside. - MSJ]